

SOUTHERN VALLEY CARDIOLOGY CLINIC

Consultation Report

Date of Service: November 02, 2026

Patient: Sarah Jenkins (DOB: 02/18/1964)

Referring Provider: Dr. H. Lin, Internal Medicine

Reason for Consultation:

Evaluation of newly onset palpitations and worsening dyspnea on exertion.

History of Present Illness:

The patient is a 62-year-old female who reports a 3-week history of intermittent "fluttering" in her chest, lasting up to 2 hours per episode. She also notes becoming easily winded when climbing the stairs at her home, which is new for her. Denies frank chest pain, syncope, or lower extremity edema.

Vital Signs:

BP: 155/92 mmHg | HR: 112 bpm (Irregular) | SpO2: 96% | Weight: 165 lbs

Objective & Diagnostics:

- Cardiovascular Exam: Tachycardic, irregularly irregular rhythm. No murmurs, rubs, or gallops.
- 12-Lead EKG (performed in clinic): Reveals Atrial Fibrillation with rapid ventricular response (RVR). No acute ST-T wave changes.

Assessment & Plan:

1. New-onset Atrial Fibrillation: Patient is at increased risk for stroke (CHA2DS2-VASc score = 3).
2. Rate Control: Initiating Metoprolol succinate 25mg daily.
3. Anticoagulation: Starting Apixaban (Eliquis) 5mg twice daily after discussing risks of bleeding.
4. Diagnostics: Ordered a comprehensive transthoracic echocardiogram (TTE) to evaluate left atrial size and ejection fraction.
5. Follow-up: Patient to return to clinic in 1 week. Instructed to go to the ER if she experiences chest pain, severe shortness of breath, or fainting.